

Chapter 2 – Understanding the Development of Attachment Bonds and Attachment Behavior Over the Life Course

Introduction

- Developmentally informed treatments for attachment disturbances are based on an understanding of what an attachment bond is and how it forms. Out of the rich and complex interplay of an infant's internal maturational processes and the interpersonal behavioral dynamics with his or her primary caregiver, an attachment bond emerges and is shaped and consolidated over time.
- The attachment bond, whether secure or insecure, creates and is reflected by an internal representation, or internal working model, of the attachment relationship and has profound influence on the child's continued development.
- Later strengths and problems in areas of self-experience, affect regulation, and capacity for intimacy in relationships can often be traced to what happened during the early processes of attachment bond formation. The treatment methods in this book reshapes an adult's internal working model (IWM) to one that is secure and repairs those areas.

I. Phase 1: The Pre-Attachment Phase (Birth to 2--3 Months)

- Automatic, reflexive behaviors occur to attract a caregiver's attention and to provide information that ideally will guide the caregiver to meet the infant's immediate needs and ensure his or her ultimate survival. The primary pre-attachment behaviors are crying, babbling, reaching, grasping, clinging, and smiling, and emerge from the innate attachment behavioral system.
- The dynamic interplay between behaviors and caregiver's responses build and shape the caregiver's attunement to the infant and thereby build the relationship. Infants' nonverbal imitative capacity also can be seen as contributing to the emerging relationship, constituting a kind of presymbolic communication.

II. Phase 2: Attachment-in-the-Making (2--3 to 6--9 Months)

- In this phase, the infant shows signs of differentiation and recognition of caregivers from others, especially the primary caregiver. The infant begins to regularly orient toward and greet the primary caregiver with verbalizations, with the newly emerged social smile, and with deliberate reaching.
- The infant begins to seek and initiate interaction, begins to develop greater control of his or her behaviors, and is more able to make deliberate responses during the interaction. Such mutual exchange can be seen as initial intersubjective communication.
- Early self-awareness emerges, between 4 and 8 months, based on growing awareness of internal bodily states. This awareness is promoted by caregiver attunement and mirroring the infant's affective expressions.
- Of note here is self-developments are promoted by relational dynamics, and then those very self-developments contribute to further refinement and development of the relational dynamics.

III. Phase 3: Clear-Cut Attachment (6–9 Months to 36 Months)

- During Phase 3, the infant's continuing maturational developments and the responses of the primary attachment figure contribute to the consolidation of this attachment bond and the development of the IWMs. Between 12 and 18 months, a child's attachment patterns become quite stable.
- Due to acquisition of locomotion, infants can seek proximity of their own accord, and sets the foundation for exploratory behavior, which is an early sign of a secure attachment bond. The attachment promoting behaviors of the caregiver continue in response to new capabilities and expressions from the infant.
- As the infant shows preferential attachment behavior toward the primary attachment figure and is more and more attentive and responsive to her or him, the caregiver is delighted and drawn to become even more attentive and responsive to the infant.
- The infant becomes able to take initiative to intentionally elicit desired responses from the caregiver which enhances the infant's experience of the availability and responsiveness of the caregiver.
- At around 8 months, facial expressive and/or autonomic fear reactions appear in response to strangers, and to some separations from attachment figures, and to some objects. The development of person and object permanence implies that the infant has and is able to respond to internal images. Though wariness and stranger and separation anxiety emerge in response to autonomic and cognitive advancements, their presence intensifies the infant's differential sense of safety with the primary caregiver.
- Between 9 and 12 months, the cognitive developmental milestone of the ability to group and categorize develops. Through and during continued interaction, the infant gets better at recognizing and now categorizing his or her affect states, and affective displays become more autonomous and directly related to discrete states of experience, which serves as a more accurate means of social communication.

The Caregiver's Role in Phase 3 Developmental Achievements

- By the end of the first year, the developmental achievements include 1) a wide range of genuine affective experiences, 2) self-awareness has continued to refine and expand due to categorical cognition, as well as awareness of "other minds" and 3) exploratory/returning to caregiver (secure base) behavior.
- The caregivers' responses contribute to the maturation of the infant's various systems and capacities, which in turn yield new infant behaviors that further engage a particular caregiver, who responds and provides experiences that foster further maturation. The foundations of affect and self-development, as well as the formation of an attachment bond, occur through this dynamic.
- The caregivers' qualities and ways of being that fosters attachment security can be separated into two categories:
 - General factors are *physical presence*, *consistency* of beneficial caregiving factors and behaviors, *reliability* of caregivers' availability and provision of caregiving, and *interest* in the infant and his or her individuality.

- The five primary behaviors that promote secure attachment are behaviors that provide 1) *protection* of the infant, 2) *attunement* to the infant's behavior, inner state, and current developmental capacities, 3) *soothing and reassurance* when the child is upset, 4) *expressed delight* in both what the child does and, most important, the infant's unique being; and 5) *unconditional support and encouragement* for inner and outer exploration.
- These five primary caregiver behaviors are important particularly for the experiences that they foster in the infant. For example, while protective behavior provides physical protection of the infant, even more important is the infant's *felt sense of safety* with the caregiver. Similarly, soothing and reassurance is an immediate response to states of distress, but more important is the infant's experience over time of a *felt sense of comfort* with the caregiver.
- These caregiver behaviors and ways of being promote the infant's sense of attachment security and provide the experiential, relational foundation for the emergence of a secure attachment bond.
- Two significant cognitive developments in the infant lead to the structuralization of these relational experiences into the *foundation of the attachment bond*, which is the formation of an IWM or map of the attachment relationship: The capacity to categorize experience, which allows the infant to link newly distinct affect states to particular caregivers, and the development of *representational cognition*, the capacity to *symbolically represent* the qualities of the caregiver relationship internally.

The Development of Representational Cognition

- Between about 12 and 20 months, the infant's cognitive system develops the capacity to link experiences to thought-based symbols. Can use specific words as symbols to indicate objects after about 12 months.
- Less visible during this period is the development of internal, symbolic representation of self and of others. By the end of the second year of life, the normally-developed child has established distinct and stable self and other representations that are in dynamic relationship internally:
 - IWMs, or "attachment maps" are internal representations of attachment relationships and determine attachment behavior during childhood and throughout adult life.
 - Central to this representation are the infant's experiences with the attachment figure, which include the caregiver's protectiveness, attunement, soothing, delight, and support, and also the infant's affective experience in response to those behaviors and ways of being.
 - Affective states become embedded in a dynamic web of internal representations of self and other. Affective states become associated with the inner experience of self and object representations.
- Feelings about the self and others become linked to their internal images, and the infant gains the ability to represent the affective *quality* of interactions. The quality of the interactions – the feelings associated with the relationship between the infant and the caregiver – are fundamental components of the attachment map.
- In aggregate over time, experiences and feelings such as safety, comfort, soothing, availability, reliability, and stability will tend to create an attachment map in which these qualities are prominent. Such an attachment map is considered secure. When over time an infant has experiences of the caregiver as unsafe,

agitating, unreliable, unattuned, rejecting, and confusing, these qualities will be prominent in the resulting attachment representation. This type of map is considered insecure.

- The representations develop an independent functional existence – they can be related to when the attachment figure is not present. The infant can have experiences and feelings about the self, about the attachment figure, and about their relationship in response to the internal representations. Further, the representations function as symbolic models of the self, others, and the relationship, and as such set up expectations about interactions.

Internal Representation, Affect Tolerance, and Affect Self-Regulation

- The infant is able to *have* various and distinct emotional experiences. but the capacity to experience a range of emotions is independent of the ability to tolerate and self-regulate them. Affect tolerance and self-regulation are dependent on the ability to evoke representations of soothing, which is dependent both on sufficient experience of being soothed when distressed and on the formation of internal representations of those soothing experiences.

- The presence of an internal representation or internal working model of an attachment figure who is present, protective, attuned, comforting, and soothing is a foundation of the capacity for affect tolerance and regulation. The more able, reliable, and consistent the caregiver is at accurately attuning to the infant's distress states and their likely sources and at providing soothing and comfort, the stronger and more stable will be the infant's internal representations of that experience and the better the infant will become at affect tolerance and self-regulation as well as self-organization.

The Emergence of Prototypical Attachment Patterns

- As the capacity for internal, symbolic, mental representation begins to form at about 12 months, the infant's experiences with the attachment figure during the first year and from then on begin to become structured as internal representations. This structuralization creates relative stability of the representations.

- The attachment prototypes A, B, C. and the addition of D (see chapter 1), are the main forms of internal working models or attachment maps that are created by the internal representation of the infant's experiences with caregivers, particularly with the primary attachment figure.

- Each prototype describes a typical pattern of behavior that is likely to occur with close others and also reflects an underlying set of expectations about the behavior of close others.

IV. Attachment Behavior During the Toddler Period (18 to 36 Months)

- The emergence of a prototypical attachment pattern during the second year, based on the establishment of an underlying internal working model, is a milestone of attachment-related development. From then until about three years, there occur no significant internal structural changes in attachment dynamics.

- Although the internal organization of attachment is fairly stable during this period, based on the stable internal working model, there are visible changes in attachment behavior that arise from other

developmental advances, such as increase in exploratory behavior as the child makes use of his or her more developed motor and cognitive abilities.

- The major developmental issue during the toddler years is “guided self-regulation”, which involves increasing self-regulation but continued dependence on the caregiver for cues, feedback, and support.

V. Attachment Behavior During the Preschool Years (3 to 5 Years of Age)

- The major developmental issue during the preschool years is increasing self-regulation independent of the caregiver.

- Other important related issues include greater self-agency with support from the caregiver, greater self-direction and self-management, and increasing peer competence.

VI. Phase 4: Goal-Corrected Partnership

- Defined as “mutual dynamic interchange toward meeting as best as possible their respective needs and goals.”

- During the third year of life children come to realize that others have their own feelings, goals, and plans, and research suggests that the *organization* of the attachment system changes significantly between the ages of 3 and 5. When learning the mother was about to initiate separation, four-year-olds would engage in negotiation to establish a shared plan and then would not be distressed by the separation.

- Increasingly, the secure child’s internal working model of the attachment relationship represents the attachment figure as available independent of proximity—when the attachment figure is not physically present, the child can call upon the representation for reassurance and soothing, and has trust that the actual attachment figure will be available in conditions of upset and need.

- This attachment map supports the four-year-old child’s growing comfort with exploring the physical and interpersonal world, behavior that promotes self-development, including interactions with nonfamiliar peers and adults.

VII. Attachment Patterns in School Age (5 to 7 Years of Age) and Middle Childhood (7 to 12 Years of Age)

- Children become less reliant on the physical proximity of parental attachment figures and more reliant on them for their general availability, particularly during times of distress. They experience more and longer separations from their parents due to school, etc. Ideally, they adapt to these separations by coming to use peers and adults other than their parents as a secure base.

- Peer perception in middle childhood is affected by the way a child perceives the parental attachment figure in early childhood. For example, if an avoidant child perceived the parents as rejecting in early childhood, he or she is more likely at this age to perceive peers as having hostile intent.

VIII. Attachment Patterns During Adolescence

- Adolescent attachment behavior is characterized by a more overarching stance toward attachment experiences -- to represent multiple divergent attachment experiences. The adolescent is able to manage the goal-corrected partnership with each parent as an attachment figure with “increased sophistication”.
- The adolescent progressively becomes more and more independent, yet still turns to parental attachment figures in times of extreme stress. Adolescent autonomy-seeking behavior can be viewed as part of the exploratory system.
- Dependencies are transferred more and more from parental to peer relationships . . . providing important sources of intimacy, feedback about social behavior, . . . and ultimately attachment relationships and lifelong partnerships. By late adolescence, certain long-term peer and intimate relationships serve the same functions as early childhood attachment relationships, especially for adolescents with secure attachment.

X. Attachment in Adult Intimate Relationships

- Certain adult romantic relationships constitute a type of *attachment relationship* in that they meet the criteria of proximity-seeking, providing a secure base, providing a safe haven, and manifesting separation distress.
- Some attachment behaviors become oriented toward different purposes. For example, proximity-seeking includes the goal of closeness for physical comfort and soothing but also includes goals based on sexual attraction. Also, the adult’s experience of having been protected as a child influences behavior towards the partner and their children.
- The main difference between early child–parent attachment and adult romantic attachments is that childhood attachments are one-sided, whereas in adult secure romantic attachment each partner mutually experiences the other as a secure base.
- The stages through which a romantic relationship becomes a deep attachment bond include pre-attachment, attachment-in-the-making (falling in love), clear-cut attachment (loving), mutual goal-corrected partnership, and emotional interdependence.
- Internal working models of adult attachment are far more complex than the IWMs of early attachment prototypes in that they include elements both of early attachment prototypes and core conflictual relationship themes accumulated across a lifetime of relationships.
- Healthy, secure adults, as compared to insecure adults, have more complex internal relational maps that can be flexibly applied and readily updated by ongoing intimate relationship experiences. People with the greatest attachment disturbance show less flexibility and updating and manifest a similar set of deactivating and/or hyperactivating strategies across most adult relationships. Treating such individuals is the subject of the rest of this book.